

bro2bro,

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Helping black health through hair

Company Name

Branding

List all Team Members

Tagline or Short

statement of purpose of
company

Tell a story.

What is the human dimension of the problem and why should we care?

What data do you have to show the size of the problem?

Show data about whether the problem is getting better or worse without your product.

Add a little bit of body text

Who is the customer of your product?

Black men around 18-40 who are seeking info on mental/physical health through trusted people

Why do they need it, to make what impact?

to help the demographic's health and reduce the amount of

How big is the market?

What are the segments of your market?

What is your customer value proposition and what was your customer validation process?

In other words, do you really know details about the people who need your product or are you guessing?

What is your solution?

Why is it better than current alternatives or the competition?

How does it work?

What, generally, does it do?

(More details of this in the demo.)

How will customers get your product?

What resistance is there and how will you overcome it?

What must happen for people who need your product to access it?

Can they afford it?

How will you reach out to more people who need your product?

How are you addressing regulatory compliance, cybersecurity and privacy issues with your product?

How will you address the challenges in the healthcare ecosystem and the market?

Show that you really understand what the drivers are in the market.

What steps are needed to enter the market and grow?

Explain how you and your teammates learned about this challenge.

What does the team think it will take to make the product financially viable/profitable or sustainable through some funding mechanism.

What people, skills and/or agents do you need to make it happen?

Does anyone on your team want to make this business happen?

If so, why? If not, why not?

Specific Questions for Each Judge in Notes from slides 8-13

Arkansas Medicaid CHW Reimbursement Status

The Legislation: Arkansas passed Act 435 (the Community Health Worker Certification Act) and Act 140 (the Healthy Moms, Healthy Babies Act). These laws officially established a statewide CHW certification program overseen by the Arkansas Department of Health (ADH) and created a legal mandate for CHW services to be reimbursed by Arkansas Medicaid and private insurers.

The Current Operational Status: While the legal pathway is authorized, full operational implementation is still in the rulemaking and federal approval phase. The state is working on the exact Medicaid State Plan Amendments (SPAs) and billing codes. Furthermore, reimbursement will be strictly tied to state-certified CHWs.

Because the direct Medicaid billing pathway is not yet fully active for general community health functions, you cannot use it as an immediate source of funding for the barbers.

Revised Framing for the Barber CHW Model

To keep the model viable right now, frame the \$500–\$1,000/month compensation as an initial B2B structure that transitions into a Medicaid-reimbursed model later.

Phase 1: The B2B / Value-Based Contract (Immediate)

The Source: The monthly stipend is paid directly to the barbershop via a Business-to-Business (B2B) contract with a hospital system, Accountable Care Organization (ACO), or a public health grant.

The Justification: The healthcare partner funds this out of their marketing, community benefit, or value-based care outreach budgets. The barbers act as trusted liaisons who drive patient enrollment, improve preventative care compliance (e.g., hypertension screenings), and reduce costly ER visits.

The health system recaptures the investment through improved quality metrics and patient acquisition.

Phase 2: Medicaid & Insurance Reimbursement (Future-Proofing)

The Source: Direct billing through Arkansas Medicaid and private health plans.

The Justification: As the Arkansas Department of Health finalizes the administrative rules for Act 435, the program will sponsor the participating barbers to go through the official state CHW certification training. Once the barbers are state-certified and the DHS billing codes go live, the partnering healthcare system will be able to bill Medicaid directly for the screenings and navigation services performed at the shop, making the stipend self-sustaining.

Proposal Tip: Frame the current lack of an active Medicaid billing pipeline as an advantage. By launching under a B2B contract now, your program will have an established network of trained barbers ready to be certified the moment Arkansas opens up direct Medicaid billing.

Gaps in our Pitch that we need to look at

No original validation data

Your evidence is entirely borrowed from BARBER-1 and UAMS. A judge will say "that's not your data." You need to frame this honestly: you're building the follow-up layer that BARBER-1 proves is needed, and your pilot will generate your own evidence.

Fix: Preempt it in Slide 3 — "We're building on proven research, and here's how we'll generate bro2bro-specific outcomes data."

Barber CHW compensation is state-specific and not confirmed for Arkansas Medicaid CHW billing varies by state. You're claiming \$500–\$1,000/month for barbers as CHWs. If Arkansas Medicaid doesn't have a CHW billing pathway, this falls apart.

Fix: Research tonight whether Arkansas Medicaid reimburses CHW services (check Arkansas DHS / Medicaid website). If yes, cite it. If not, frame compensation as coming from the B2B contract, not Medicaid billing, until the pathway is established.

Answered on 9

The Trustee feature has a privacy risk

A judge (especially the BCBS or VA judge) might ask: what if a Trustee is an abusive partner, an employer, or someone who could use the health information against the man? This is a real scenario.

Fix: Add one line in Slide 6: "Trustees see only what the user explicitly chooses to share. There is no automatic disclosure. The feature is entirely opt-in and revocable."

UAMS partnership is aspirational, not confirmed

If a judge (especially Eric Peterson from UAMS BioVentures) asks "are you partnered with UAMS?" and you say no, it undercuts everything. Saying you're "building on" their research is fine but needs to be framed carefully.

Fix: Frame it as "we are building the commercial vehicle for their research, and the next step is a formal partnership conversation — one we're hoping to start in this room."

The clinical handoff mechanism is vague

You say the platform "connects men to care" but the specific mechanism — how an at-risk man goes from a text conversation to an actual appointment — is not spelled out in the pitch.

Fix: Be specific in Slide 4. "When a risk flag is detected, the platform surfaces two local providers and an SMS booking link. If he texts 'book,' it sends a scheduling request directly to that clinic's system."

LLM data lifecycle — Ningning Wu will ask this

Wu's research is specifically on LLM data lifecycle risk detection. She will ask what happens to conversation data, whether PHI enters the model context, and how you govern the AI. If your tech team hasn't addressed this architecturally, it will show.

Fix: Brief the pitch team tonight on the specific answers (see Wu's Q&A above). If you don't know your AI's data retention policies, look them up — every major LLM API publishes them.

Peer mentor liability and quality control

You mention peer mentors but don't address how they're screened, trained, or liability-protected. A clinical judge will ask what happens if a peer mentor gives harmful advice.

Fix: Add one line in Slide 4: "Peer mentors share personal experience — they do not give medical advice. They are screened, trained in scope boundaries, and covered under the platform's terms of service, which explicitly distinguishes peer sharing from clinical guidance."

The tagline may undercut your multi-touchpoint story

"Helping black health through hair" implies barbershop-only. If a judge asks "what about Black men who don't go to barbershops?" — rural men, older men, young men who cut their own hair — your tagline works against you.

Fix: Either update the tagline ("...through community") or address it explicitly in Slide 4: "The barbershop is our launch channel — but bro2bro works through any trusted community setting, and via SMS for men who can't access any of them."